Lakeview Community Hospital
Department of Internal Medicine

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Discharge Summary

Patient	Thomas R. Halvorsen	**MRN**	00482913
Date of birth	7 February 1958	**Sex**	Male
Admitted	9 October 2026	**Discharged**	16 October 2026
Ward	Medical Ward 3B	**Consultant**	Dr. Ingrid Solberg
Discharged to	Home, with community nursing	**Family doctor**	Dr. Paul Mercer, Eastside Medical Practice

Diagnoses

1. Community-acquired pneumonia, right lower lobe (primary diagnosis)
2. Acute kidney injury, stage 1, resolved
3. Type 2 diabetes mellitus
4. Hypertension

Presenting complaint

Mr Halvorsen presented to the emergency department with a four-day history of productive cough, fever and increasing shortness of breath. On arrival his oxygen saturation was 89% on room air and his temperature was 38.9 °C.

Investigations

Test	9 Oct	12 Oct	15 Oct	Reference range
Haematology				
Haemoglobin (g/L)	132	128	130	130–170
White cell count (×10⁹/L)	16.8	11.2	8.4	4.0–11.0
Biochemistry				
C-reactive protein (mg/L)	186	74	21	< 5
Sodium (mmol/L)	134	137	139	135–145
Potassium (mmol/L)	4.9	4.3	4.1	3.5–5.3
Creatinine (µmol/L)	148	112	96	60–110
HbA1c (mmol/mol)	62	–	–	20–41

A chest radiograph showed consolidation in the right lower zone without effusion.

Hospital course

He was treated with intravenous amoxicillin and oral clarithromycin, and was switched to oral antibiotics on day three once his fever had settled. His renal function improved with intravenous fluids and temporary withholding of ramipril and metformin.[1] He was mobilising independently and maintaining saturations above 94% on room air at the time of discharge.

[1] Both medicines were restarted on 14 October 2026 after creatinine had returned to baseline.

Medication on discharge

Medicine	Dose	Route	Frequency	Duration	Status
Amoxicillin	1 g	Oral	Three times daily	Until 19 October 2026	New
Clarithromycin	500 mg	Oral	Twice daily	Until 19 October 2026	New
Metformin	1 g	Oral	Twice daily	Ongoing	Restarted
Ramipril	5 mg	Oral	Once daily	Ongoing	Restarted
Atorvastatin	20 mg	Oral	At night	Ongoing	Unchanged
Paracetamol	1 g	Oral	Up to four times daily	As needed	New

Follow-up

- Renal function and potassium check in one week at the family doctor's practice.
- Repeat chest radiograph in six weeks, to be requested by the family doctor.
- Diabetes nurse review of glucose control within one month.

Advice to the patient

Complete the full course of antibiotics even if you feel better. Seek urgent medical help if you become more breathless, develop chest pain or cough up blood.

Dr. Amir Haddad, resident physician, on behalf of Dr. Ingrid Solberg, Consultant Physician
Electronically signed on 16 October 2026 at 14:32
Copy to: Dr. Paul Mercer, Eastside Medical Practice